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NAME: TEST PATIENT ZETA **AGE/SEX: 58 / M**
REFD. BY: TEST REFERRING DOCTOR **DATE: 15/10/2026**
UHID: TEST-006 **ACCESSION: ACC-MRI-CS-002**

MRI CERVICAL SPINE

CLINICAL HISTORY:

Chronic neck pain with bilateral upper extremity paresthesias

TECHNIQUE:

Multiplanar, multisequence MRI of the cervical spine was performed without intravenous contrast.

MEASUREMENTS:

Measurement	Value
C2-C3 Canal AP	13.2 mm
C3-C4 Canal AP	12.8 mm
C4-C5 Canal AP	11.5 mm
C5-C6 Canal AP	10.2 mm
C6-C7 Canal AP	12 mm

FINDINGS:

Straightening of normal cervical lordosis. Multilevel degenerative changes are noted. At C4-C5, there is a disc osteophyte complex causing mild central canal narrowing and mild bilateral neural foraminal narrowing. At C5-C6, there is a disc osteophyte complex causing moderate central canal narrowing and moderate bilateral neural foraminal narrowing. The cervical cord demonstrates normal signal intensity, though it is closely apposed to the posterior disc osteophyte complex at C5-C6. The visualized soft tissues of the neck are unremarkable.

IMPRESSION:

1. Straightening of cervical lordosis.

2. C4-C5 disc osteophyte complex with mild central canal and mild bilateral neural foraminal narrowing.

3. C5-C6 disc osteophyte complex with moderate central canal and moderate bilateral neural foraminal narrowing.

RECOMMENDATION:

Clinical correlation for cervical spondylosis with possible cervical radiculopathy. Consider EMG/NCS if symptoms progress.

Dr. Sugandha Priyadarshini
MD (Radiodiagnosis & Medical Imaging)

Radiological diagnosis is not always conclusive & often vary with clinical course of the disease or response to treatment. This report is not for medico-legal purpose.